

SECTION | 5

Experiential Exercises:

Reenactments, Rescripting, and Repair

Protectors who are in the habit of fighting, fleeing, or freezing in pursuit of safety interfere with a couple's ability to attune and be the mutually loving resource each of them needs. Experiential exercises, which involve less talk and more focus on the body, are a great way for couples to deepen their internal and external understanding by doing something different. We help them move beyond considering change to having new, powerfully integrating experiences.

The experiential exercises here involve reenacting and rescripting a painful interchange. As with any model of therapy, practice highlights further possibilities. Therefore, once you have absorbed the basic tools of the IFIO approach, we encourage you to trust your intuition and develop more experiential exercises.

EXPERIENTIAL PLAY

Creative rescripting helps couples deactivate their ANS, repair misunderstandings and miscommunications, and change entrenched relational patterns. It also buoys hope and reveals more material to explore. The willingness of partners to try a new behavior depends, in our view, on their protectors being willing to unblend and concede that a new behavior might help. Inner differentiation gives each partner more space to do the essential U-turn and notice their own parts' dilemmas. Once the Self is differentiated from their parts, we want each of them to be more involved with their own internal systems. Ironically, this inner attentiveness helps them accept and be kind toward their partner's struggles. Kindness inside launches a beneficial cycle and prepares partners to be kind to each other by interrupting hurtful patterns on their own between sessions.

With experiential exercises, we ask partners to move out of their comfort zones and try something different. These exercises slow everything down and help partners embody a dilemma that they can work out in vivo with the therapist's help. We introduce experiments when we judge that the couple's mutual trust level is high enough and we feel confident that an experiment will be instructive.

Borrowing from Gestalt therapy, the following therapist handout contains two experiments that guide clients to learn by doing (Roubal, 2009). These experiments are creative and adventurous. They can be simple or complex depending on the situation and the couple's level of comfort. They aim to help each partner unblend, see how they behave in conflict, and—by embodying their partner's parts—understand and empathize with their partner. Once the experiment is launched, we go with the flow. Our guiding instruction is to hold agendas for specific outcomes lightly so we can be open, curious, and Self-led. The experience helps us understand strong feelings in the present, and it may reveal important issues for future attention.*

Therapist Worksheet

EXPERIMENTING WITH EXPERIENCES:

Re-Enactment

To set up the following two experiments, begin by asking the couple if they are interested in experimenting, and describe what you have in mind. Create safety during this process by noticing first if you have the confidence to stay present for whatever happens. If not, help your parts unblend. Then get permission from the couple to try something new and welcome resistant parts.

During an experiment, pay attention to moments when a partner's inquiry could deepen, and encourage them to express vulnerability. Help them anchor their experiences by continually asking what they notice and what's happening in their bodies.

Finally, leave time for processing individual and relational experiences.

Experiment #1: Doing a U-Turn

1. Invite the couple to choose and describe an incident to focus on. Choose an incident that evoked strong sensations, feelings, and thoughts in one or both partners. Before proceeding, help them decide what they can agree on about the event, and remind them that memory is inaccurate, especially when coupled with strong feelings.
2. Invite the partners to stand and move around the room if they're willing.
3. Negotiate whose experience will be attended to first. We will call this person Partner A.
4. Invite both to go inside (do a U-turn) and ask protectors to unblend so each partner can find the parts who were involved with the event in question.
5. Check to see if Partner B is aware of their parts and can stay present to listen.

6. Then direct your attention to Partner A and ask, “What did Partner B say or do at the time that triggered this cascade of sensations, feelings, and thoughts?”
7. Tell Partner A that you are going to ask Partner B to reenact that triggering behavior, and ask Partner A to notice:
 - Their body
 - What they hear their parts saying internally
 - Their feelings
 - The part who seems most vulnerable in this situation
8. Then ask Partner B to reenact the triggering behavior.
 - Check in with Partner A during this time. Did they notice the vulnerable part under their reactivity? Take a moment to explore its hopes, fears, and needs.
9. Check in with Partner B:
 - What was it like to act that way again?
 - What did they notice about their parts?
 - What feelings come up now?
10. Now (if there’s time) or in the next session, reverse roles and repeat the sequence.

Experiment #2: Going Deeper: Becoming the Other

With the aim of helping partners truly understand each other and gain a broader perspective on their dynamic, this experiment goes beyond role reversal by asking each partner to use their imagination to embody the other partner and feel into the other’s parts. This experiment begins with the initial steps of experiment #1:

1. Ask Partner A, “What did your partner say or do at the time that triggered this cascade of sensations, feelings, and thoughts?”
2. Invite Partner B to reenact the trigger behavior while Partner A notices:
 - o Their body

- o What they hear their parts saying internally
 - o Their feelings
 - o The part who seems most vulnerable in this situation
3. Check in with Partner B:
 - o What was it like to act that way again?
 - o What did they notice about their parts?
 - o How do they feel now?
 4. Now, instead of simply reversing roles, ask Partner A to imagine *being* Partner B. Slow this way down. Explain that Partner A will repeat the triggering behavior while noticing their body, what they hear inside, and what they feel. Then invite Partner A to enact the triggering behavior.
 5. Now explore what Partner A noticed while doing the behavior, and ask Partner A to speak for the wishes and needs of Partner B's parts.
 6. Next, ask what Partner B noticed about being in Partner A's shoes, and ask Partner B to speak for the wishes and needs of Partner A's parts.
 7. Ask each partner to speak for the experience of being in the other's shoes:
 - What have they learned?
 - What surprised them?
 - Did they notice any projections? That is, anything that belonged to them rather than the other person?
 - Take time to explore how this might inform future inquiry.
 8. Finally, invite them to rescript their original interaction to see how it goes now.
 9. Take time to note new insights before stopping.

BECOMING THE OTHER

The following vignette illustrates a rescripting sequence in which a couple first reversed roles and then became each other. The couple, Kim and Dave, had been married for 17 years. They were cisgendered, heterosexual, in their fifties, and had two school-age children. Kim, who was biracial (a European-American mother and an African-American father), was raised in England. Dave, who was African American, was raised in the United States. Kim's job as a lawyer required her to travel at least five days a month. Dave, who was a tenured professor at a local university, did not travel regularly.

Kim seemed comfortable in therapy sessions, given that she had been in therapy in the past and her mother was a psychologist. Dave, on the other hand, had never been in therapy and was initially concerned about his reputation at the university and the stigma of having mental health issues. They expressed a great deal of goodwill toward each other, parented together well, reported good communication, and generally felt satisfied in their relationship. But they had not been able to break a pattern of quarreling whenever Kim returned home from a business trip. These fights were quick and painful.

The first few therapy sessions, which were rich and proved helpful for everyone, focused on their feelings about working with the therapist, a cisgendered, lesbian, European American, who grew up in the United States. The next focus was on hearing from Dave's parts, who were wary of therapy. This went well. Both partners found that parts language helped their protectors unblend so they could notice vulnerable exiles, and they appreciated learning how their nervous systems were functioning during stressful conversations. Dave learned that he had parts who tended to fight and then flee, while Kim had parts who tended to shut down quickly. They were quick to use the tools the therapist taught them to help these protectors unblend. The following session illustrates an experiment in which they reenacted and then rescripted a typical fight.

Kim: We're still struggling with this thing we get into when I get back from a trip. It's sad. I feel so ready to come home. I've missed Dave and the kids. And then this difficult dynamic happens. I'm exhausted by the travel, and I don't cope with it well.

Therapist: And this is where you want to focus today?

Kim: It is.

Therapist: [*to Dave*] Okay with you?

Dave: Yes. It's fresh in our minds. Kim just got back from being on the West Coast for longer than usual.

Therapist: [*to Dave*] Any more you want to speak for about that?

Dave: I feel pretty upset. I notice that I felt criticized and dismissed. And then I got dismissive.

Therapist: Who would like to start unpacking this dilemma?

Kim: [*to Dave*] Since you're upset right now, do you want to start?

Dave: Sure.

Therapist: [*to Dave*] Let me just remind you that you'll be speaking for your experience and that memory is distorted by strong feelings. [*turning to Kim*] And you'll have the chance to fill in when Dave finishes.

Kim: Fair enough.

Dave: Okay. So, as we've discussed, my sensibility about housekeeping is different from Kim's. As far as she's concerned, when she's away and I'm in charge, things go to hell in a handbasket. I'm disorganized, and the kids are all over the place, so the house ends up being chaotic. I do my absolute best to get everything together before she gets home because I know she will get tweaked when things aren't perfect. But the minute she walks in, she notices the chaos rather than what I've done to pull it together. She seems frustrated; I feel dismissed. I get pissed off and remove myself. I take it really personally.

Therapist: [*to Kim*] Want to add?

Kim: Well, I hear that, and I know all that. But his story is that I want things to be perfect. And while it's true that I spot it when things are out of place, and I do get tweaked or start micromanaging him and the kids, his response is unkind. He can be mean. I'm exhausted from travel and putting out about a 150 percent of my energy for days on end. I come home looking forward to some rest and peace. So I don't like this part of me, the one who sees the chaos and forgets the effort (thank you Mother!), but I also don't like Dave's response. I get pissed and kind of lecture-y with him. He sulks off, and I storm off. Then I shut down and hear myself saying things like "Why do I even bother coming home?" I guess I just feel lonely.

Therapist: This pattern is recognizable, yes? [*both nod*] I have an idea. If you're game, let's unpack this scenario in a different way right now. Let's do it over in slow motion so you can discover what's under your reactions to each other and feel into each other's experiences. This exercise is like tracking your fights and understanding what your protectors do or say, but it's more in depth.

Introducing the couple to the idea

Kim: Can you say more?

Therapist: I'll be asking you to stand and move around. [*Dave winces*] Of course, you don't have to do it that way. I'll give you choices.

Dave: No, I'll try. If it's too strange, I'll let you know.

Therapist: Great. I'll invite you to go back to what happened earlier this week and reenact it from when Kim comes in the door. I'll ask you both to notice all the parts who are engaged in the situation, and we will pause as needed to ask them to unblend.

Creating safety by going into more detail about the exercise

Kim: I'm up for it. Dave?

Dave: Sure.

Therapist: Although I have an idea about how this will go, we're exploring, and, as you've probably noticed, anything can happen. So we'll see where it takes us. Okay? [*they nod*] Stand up and place yourselves anywhere in the room. Dave, can we start with you?

Dave: Sure.

Therapist: [*to Kim*] For now, I'm going to coach you. Let's start with noticing any parts who may not want to hear from Dave and asking them to unblend. Then take a scan of your body, and make sure you have enough space to listen.

Offering help

Kim: [closes her eyes, takes a moment inside, then opens her eyes] Believe it or not, I am pretty available. I think we both play a part in this, and I feel embarrassed about being so critical.

Therapist: You have a part who can be critical?

Reframing with parts language

Kim: I do.

Therapist: I am going to ask you to come in the room and do or say what you normally do or says that gets to Dave.

Kim: [stands at the opposite end of the room and walks toward Dave] Hi honey!

Dave: Welcome home!

Dave gives her a hug. Kim simulates taking off her coat and searching for a hanger. Then she turns to Dave and speaks in an irritated tone.

Kim: Why are there these shoes dumped all over the closet? And why are there no hangers for my coat?

Therapist: Dave, take a moment to notice what's happening in your body.

Dave: Yup, there it is! The part who gets pissed.

Therapist: What do you hear it saying? Take your time.

Dave: It doesn't matter how hard I try. I can't get it right! I have been working tirelessly to get this house cleaned up for Kim. It's never going to happen.

Therapist: This part is afraid it's never going to get this right.

Reframing with parts language

Dave: Yes.

Therapist: And what does that mean about you?

Dave: I'm inadequate.

Therapist: And when you feel inadequate what happens?

Dave: I get mad.

Therapist: And then you notice an impulse to move away from Kim?

Dave: That's right.

Therapist: Can you feel those urges to fight and flee in your body?

Naming the autonomic response

Dave: I feel those energies running right through me.

Therapist: [to Kim] How are you doing?

Kim: Good. I just want to listen right now.

Dave: You're not tweaked by this?

Kim: Not at the moment.

Therapist: [to Kim] Are you available to hear more?

Kim: I sure am. Though I want to make sure Dave will listen to me at some point.

Therapist: Of course. We'll reverse roles later today or, if we don't have time today, in another session. [to Dave] You have a lot of energy in your body. Would it help to move?

Reassuring protectors

Dave: As a matter of fact, yes! [*he paces and shakes his arms, then stops and takes a deep breath*]
Okay I feel better. I still notice a strong desire to get away. It's weird to pay this much attention to my body.

Therapist: [to Kim] What's happening with you?

Kim: I can relate to having all that energy in my body and then getting stuck trying to figure it out mentally.

Therapist: Dave, can we keep going with your parts? [*Dave nods*] As you pay attention to the sensations in your body, can you find what's underneath all the energy?

Dave: [to Kim] I feel hurt. That's the bottom line. I just don't get why some correction or annoyance has to be the very first thing out of your mouth when you get home.

Therapist: It makes sense that you want to ask Kim this question, Dave, but first can you speak more for that hurt? We'll get to your questions.

Dave: I feel the hurt. *[to Kim]* I am trying to take care of you by doing what doesn't come naturally to me—but it's never enough, and that hurts. This is a vulnerable thing to admit. I've worked so hard in so many areas of my life to get it right, but the place where it's most important, I always fail. I don't think you get how hard I try. You don't see what I do.

Therapist: *[to Kim]* What's happening?

Kim: I feel guilty. I do have a part who feels out of control about this, but it's not that you aren't enough.

Therapist: *[to Kim]* Given what Dave has revealed about his childhood, does the burden he's describing of never getting it right make sense to you?

Kim: Yes certainly.

Therapist: *[to Dave]* What's it like to hear that?

Dave: Good, actually.

Therapist: Dave, do you have a request for Kim?

Dave: I do. I want you to greet us before you start talking about what annoys you and before you start moving things around to take care of yourself. I want you to see what's right before you look for what isn't. Does that make sense?

Kim: It really does, Dave. We joke around that I have an OCD part, but I think we're onto something. Maybe not OCD per se, but something like that. It's my struggle. I am committed to working on it. I don't want to hurt you or put the kids in the crossfire.

Therapist: Right now, Dave, what's it like for your parts to be seen and heard?

Dave: Way better. Thank you. *[turns to Kim]* Thank you, Kim, for hearing me and agreeing to work on this.

Therapist: We have some time left. *[to Dave]* Are you available to learn about Kim's experience?

Dave: Very.

Therapist: This time I'm going to ask you to do something a little different. I want you to reverse roles. To get a sense of what goes on inside Kim, you will stand in her shoes. Okay?

Telling them the plan and asking for permission

Dave: Sounds okay to me.

Kim: I love this idea!

Therapist: Stand up again, Dave. Feel your feet on the floor, close your eyes, and find your inner Kim. Start with what she was feeling when she arrived home.

Dave: Tired, happy to be home, looking forward to seeing us. [*Kim nods*] So I come inside, get a hug from Dave, open the closet to hang my coat up, and my heart sinks. In my head I hear, “What about me?” The shoes are a jumble on the closet floor, and I can’t even find a hanger. I’m tired, and now I’m thinking about having to clean up after everyone the minute I get home. I feel disappointed and lonely.

Therapist: Great, Dave. You can open your eyes now. What was that like for you?

Dave: Sad. She comes home tired, but she wants to be home again—and then it’s ruined for her.

Therapist: Okay to go a little deeper? Close your eyes again and find her parts.

Deepening the inquiry

Dave: [*closes his eyes and is quiet for a few seconds*] I sense a need to be taken care of. And maybe some feelings of being overwhelmed and out of control. [*opens his eyes and looks at Kim*] Am I getting this right?

Kim: Exactly right. Then my irritated part steps in to hide that.

Dave: And then my irritated part reacts.

Therapist: Kim, what’s it like to hear Dave now?

Kim: Great. Isn’t it amazing that we feel just about the same way?

Therapist: And since these internal conversations drive your external conversations, how would it be to speak for that with each other? And even make a regular exercise of putting yourselves in each other’s shoes?

Dave: I really like this exercise. I got to something important today. I’d be willing to try it at home

Therapist: Having stood in Kim's shoes, what do you think she needs most?

Dave: To be understood.

Kim: And for you to be kind when you call me on something.

Dave: Makes sense. I guess that would involve me not feeling victimized.

Therapist: That's right. My invitation is for you to cultivate your open heart even when your partner is blended with a reactive part. I understand that speaking for underlying needs can be challenging when one or both of you feels reactive. But as you gain more understanding about your inner dilemmas, you'll recognize your partner's dilemmas too, and compassion will take over from all that reactivity. Let me say just one last thing today. I hear the vulnerability under your protectors. A part of you, Kim, says, "What about me?" And a part of you, Dave, says, "Do you see me?" These are important parts. We'll come back to them next time.

Providing an overview and inviting them to explore new material in the future



As this example illustrates, experiments that move beyond the basic steps of IFIO enliven sessions with couples. This experiential exercise involved replaying a troubled interaction, slowing both partners down to do a U-turn and observe their parts, doing the scene over, and, finally, doing it again in reversed roles. It allowed them to explore their inner world, talk about what they discovered safely, and try new behaviors. Reenactments and role reversals develop a couple's awareness of alternatives while building their understanding and self-acceptance (Yaniv, 2018). The experience can lead to spontaneous repair. Additionally, the information we glean from the exercise gives direction for future sessions.

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